

Healthy People 2030 (in 2026), Access to Primary Care

Literature Summary & References | Source: odphp.health.gov

About This Literature Summary

This summary of the literature on Access to Primary Care as a social determinant of health is a narrowly defined examination that is not intended to be exhaustive and may not address all dimensions of the issue. Please note: The terminology used in each summary is consistent with the respective references. For additional information on cross-cutting topics, please see the [Access to Health Services](#) literature summary.

Related Objectives (4)

Here's a snapshot of the objectives related to topics covered in this literature summary. [Browse all objectives](#) .

- [Reduce the proportion of people who can't get medical care when they need it — AHS-04](#)
- [Reduce the proportion of people who can't get the dental care they need when they need it — AHS-05](#)
- [Increase the proportion of people with a usual source of care — AHS-07](#)
- [Increase the ability of primary care and behavioral health professionals to provide more high-quality care to patients who need it — AHS-R01](#)

Related Evidence-Based Resources (4)

Here's a snapshot of the evidence-based resources related to topics covered in this literature summary. [Browse all evidence-based resources](#) .

- [Comprehensive Primary Care Plus](#)
- [Creating Patient-Centered Team-Based Primary Care](#)
- [Improving Access to Oral Health Care for Vulnerable and Underserved Populations](#)
- [TeamSTEPPS® for Office-Based Care](#)



Healthy People 2030 organizes the social determinants of health into 5 domains:

1. Economic Stability
2. Education Access and Quality
3. Health Care Access and Quality
4. Neighborhood and Built Environment

5. Social and Community Context

Literature Summary

The National Academies of Sciences, Engineering, and Medicine (formerly known as the Institute of Medicine) define primary care as “the provision of integrated, accessible health care services by clinicians who are accountable for addressing a large majority of personal health care needs, developing a sustained partnership with patients, and practicing in the context of family and community.”¹ A primary care provider is usually an internist, family physician, pediatrician, or non-physician provider (e.g., family nurse practitioner, physician assistant).^{2, 3} Research shows that access to primary care is associated with positive health outcomes.^{2, 4}

Primary care providers offer a usual source of care,³ early detection and treatment of disease,⁴ chronic disease management, and preventive care.² Patients with a usual source of care are more likely to receive recommended preventive services such as flu shots, blood pressure screenings, and cancer screenings.^{3, 5, 6} However, disparities in access to primary care exist, and many people face barriers that decrease access to services and increase the risk of poor health outcomes.⁷ Some of these obstacles include lack of health insurance,^{7, 8, 9} language-related barriers,¹⁰ disabilities,¹¹ inability to take time off work to attend appointments,¹² geographic and transportation-related barriers,¹³ and a shortage of primary care providers.¹⁴ These barriers may intersect to further reduce access to primary care.

Lack of health insurance decreases the use of preventive and primary care services and is associated with poor health outcomes.^{7, 8, 9, 15, 16} Individuals without health insurance may delay seeking care when they are ill or injured, and they are more likely to be hospitalized for chronic conditions such as diabetes or hypertension.^{4, 7, 8, 9} In addition, children without health insurance are less likely to get vaccinations, a routine primary care service.¹⁷ Overall, having health insurance increases the use of health services and improves health outcomes.^{18, 19}

Only offering primary care services in English can negatively impact access to primary care and screening programs for patients who speak a language other than English.¹⁰ For example, a study found that Hispanic children who do not primarily speak English and immigrant Hispanic children are more likely to lack a usual source of care compared to non-immigrant Hispanic individuals in English-speaking households.¹⁰ Similarly, a study examining health quality of life outcomes in older populations discovered that older immigrants with limited English proficiency had less access to health care than older adults who speak English fluently.²⁰

Limited provider office hours and availability can be barriers to accessing primary care.^{21, 22} Many primary care providers do not offer services during typical off-work hours, posing barriers to workers without sick leave benefits.¹² One study found that even when workers were provided with sick leave, some did not take time off to receive primary care because they still feared they would lose wages.¹² Additionally, primary care provider shortages and extreme demand often make it challenging for patients to get an appointment.¹³

Factors such as access to transportation, travel distance, and the supply of primary care providers can also limit people's ability to get primary care.¹³ For example, rural residents may need to travel long distances to get primary care and thus may be less likely to seek preventive care such as vaccinations.¹³ In addition, rural communities tend to have fewer providers than urban communities;¹ this relative shortage of providers may make it harder for rural residents to access primary care.¹³

Primary care is critical for improving population health and reducing health disparities.⁴ Therefore, addressing barriers to accessing primary care may help reduce disparities and reduce the risk of poor health outcomes. For example, the National Health Service Corps supports the work of primary health care clinicians in areas of the United States with limited access to care, also called health professional shortage areas (HPSAs).²³ Digital solutions like telehealth can also improve access to primary care by reducing barriers related to transportation and expanding the ability to offer services in languages other than English.²⁴

Further research is needed to better understand barriers to primary care, offer support to primary care providers, and develop interventions that expand primary care access. This additional evidence will facilitate public health efforts to address access to primary care as a social determinant of health.

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